

This was a final project for Introduction to Medical Anthropology that involved interviewing someone about their personal experience with medicine and then translating that to a project (format was open to personal choice). I decided to write a creative piece and analyze it to demonstrate the ways in which it relates to both the interview I conducted as well as content from the class.

Markedly Unfathomable: A Different Kind of Medicine Story

Markedly Unfathomable

I am tired.

That is a phrase often reserved for those who push themselves to the point of collapse, a futile effort toward achieving the contracted “fulfillment” society has manufactured.

In today’s modern economy, falling prey to the temptations of gambling one’s very existence is easy. However, frugality has become a sinful virtue of mine. In my early days, I dreamed of ease. Despite my sufferings, I beamed at the oracles of health, hoping that one day I too will be saved by their miracles. Their kind poisoned me with their deceitful promises, yet I remained hopeful. As if I were a hovel-dwelling creature, I dared to confer with my perceived saviors. In my pursuit of understanding, I soon learned of their aversion to the unknown—I have reason to suspect that they would sooner distort my story into that of a mindless beast than ever truly listen to my tale. I finally understood that I do not fit into their classifications of human. Am I truly so different, so abhorrent that humanity does not reach me?

At first I startled back, unable to believe that it was indeed I who was reflected in the mirror; and when I became fully convinced that I was in reality the monster that I am, I was filled with the bittersweet sensations of despondence and mortification. Alas! I did not yet entirely know the fatal effects of this miserable deformity.¹

As I continued on, doubts of my humanity grew heavier. Is my dejected state not natural for a life beyond saving? Am I unnatural? Suffering is central to human existence, yes, but in my attempted interactions with humankind I fear that my suffering may be of a kind markedly unfathomable to them. Why, then, am I expected to continue living as they do?

I was benevolent; my soul glowed with love and humanity; but am I not alone, miserably alone?²

The only repose guaranteed in life is death.

I am tired, and I long for rest.

¹ Mary Shelley, *Frankenstein*, pp. 114-115.

² Mary Shelley, *Frankenstein*, p. 101

ANALYSIS

This story was inspired by the lovely human I interviewed, but was also inspired by my own experiences, as well as tales told by others—human and “inhuman”. My decision to write a short story derived from the prevalence of themes of alienation found throughout the interview. As with most art, literature is an outlet for expression. Novels from the 19th and 20th centuries, especially those considered classics, are especially notable for their common themes of alienation. When considering those themes within the specific context of mental health and biomedicine, two novels that I had previously read stood out to me, and I decided to incorporate them into the story.

The foundational influence for this short story is the work of 20th century Japan’s literary giant Osamu Dazai, specifically *No Longer Human*, one of his most famous novels. *No Longer Human* is a first-person narrative telling the story of Oba Yozo, a man who feels he is too troubled to qualify as human. Dazai’s novel is what influenced me to choose a first-person narrative to explore the feelings of alienation that often result from chronic mental health struggles, as well as the suicidal ideation that often accompanies such an existence. The unreliable narration provided by Yozo perfectly conveys how the mind can negatively contort one’s self-perception into absolute truths, completely disregarding any outside perceptions that may be more accurate, or at the very least more forgiving. This story is crafted similarly in that the reader is only provided the narrator’s thoughts and feelings, and the reader may even begin to wonder if perhaps the narrator is indeed such an outcast. I also intended for the first-person narration to feel more personal, with the questions inviting the reader to consider the narrator’s case.

Another literary work I incorporated into this story is Mary Shelley's *Frankenstein*. The way Frankenstein's creature recounts to his creator his descent into "wretchedness" particularly stood out to me, as his rationale closely resembles the thought process that often leads to severe feelings of alienation. I was initially only going to incorporate allusions to the novel, but I quickly realized that I wanted direct dialogue, hence the two italicized bodies of text embedded in the story. I incorporated them in such a way for the excerpts to have a feeling of them being echoing thoughts of the narrator, simulating thoughts lingering in the back of the mind.

In order to adequately explore the incorporation of these novels, as well as the incorporations of and connections between the interview and class readings, here is a careful analysis of the story:

"I am tired.

That is a phrase often reserved for those who push themselves to the point of collapse, a futile effort toward achieving the contracted 'fulfillment' society has manufactured.

In today's modern economy, falling prey to the temptations of gambling one's very existence is easy."

This first part of the story comments on the experience of living in today's capitalist society, specifically on the way people are expected to push themselves to an unhealthy extent in order to survive, or to—at the very least—"keep up". The "gambling" mentioned is the risking of health for the sake of "progress" as defined by capitalism. Capitalism has even constructed the narrative that pushing yourself to a debilitating degree is the practice of having a "good work ethic". Individuals often worry that they are never doing enough, and that they will "fail". As said by my interviewee:

"I feel like capitalism ties in with me being valued at what I produce, so without my ADHD meds it becomes a lot harder for me to produce work quickly and efficiently and on time...if I can't focus, and if I can't get done my homework, and if I can't keep a consistent train of thought, then how am I supposed to get my higher education and then get a higher paying job and fulfill these things that are expected."

Capitalist expectations and fear of failure often lead to feelings of inadequacy and, when those feelings are prolonged, feelings of alienation are not far behind. As Elliott Fukui states in his article "Stay Mad: A Love Letter to QTBIPOC Psychiatric Survivors", "I am a failure because I do not meet the bar for being considered a person under capitalism". As mentioned by Kai Chang Thom in her article "Belief in Mental Health", mental health is either defined by an "absence of mental illness" or in some cases are "suspiciously oriented toward "productivity" and individual achievement". Thom then elaborates, asking, "aren't those the very values that make late-stage capitalism so very destructive to our well-being?".

The line **“frugality has become a sinful virtue of mine”** is a declaration of resisting capitalism by not pushing oneself to drastic lengths for the sake of eventual “fulfillment”, that “frugality” refers to not sacrificing oneself for that goal. However, while prioritizing one’s rest over capitalist gain is generally a healthy decision, the narrator’s “frugality” within the context of this story refers to resigning oneself to a state of deterioration. Instead of a better goal, there is no goal. This line is intended to reflect experiences of depression.

“In my early days, I dreamed of ease. Despite my sufferings, I beamed at the oracles of health, hoping that one day I too will be saved by their miracles. Their kind poisoned me with their deceitful promises, yet I remained hopeful.”

This part refers to the faith people often place in the “miracles of medicine”. My interviewee shared that as she was struggling as a child, she hoped that medicine could help ease her suffering, but quickly realized that a supposed “cure” may not be possible for her:

“[W]hen I first went to the psychiatrist and he told me when I was 12 years old, like ‘oh look I can fix your anxiety with this magical medication’ and you as a child don’t really know the consequences that can come with taking this medication, I was like ‘this is amazing! I’m never going to be anxious again! I’m

so excited for that!', but then, when the reality sinks in of 'oh no, I'm gonna have to deal with this forever', that kind of sucks to have to—it feels like grieving something that you thought you weren't going to have to grieve anymore."

Because of the way biomedicine measures success via "fixing problems", it is easy for an individual to feel disheartened when biomedicine does not "fix" them. However, as Thom proposes in her article, "mental health was invented to sell us something... 'mental health' itself is a shapeshifting fantasy, a dream with no substance that fades even as we are told we are grasping it". This concept extends to the way that modern society promotes that the "mentally *unhealthy*...are responsible for managing [their] own illness in private, for [their] own failure to recover, for [their] inability to be and feel and do what is needed of us". Mental health treatment is often helpful for those who need it, but it is dangerous to promote it as the solution to one's mental struggles, especially as it works within the violence of capitalism. Not only is it dangerous because of the way it can worsen one's self-perception, but it also perpetuates iatrogenic harm. With biomedicine being as egotistically rigid and entrenched in capitalism as it is, medication is sometimes irresponsibly prescribed. As my interviewee experienced in her time seeking treatment for her chronic mental health struggles,

"[I]t just felt like they just wanted to get me to quit coming to them, 'cause they just would try and like instead of actually trying to understand what was making my brain work the way it was and making it literally impossible for me to function. They were like, 'oh, well, if we just get you out of here, then maybe you'll eventually quit coming or one of these will just happen to stick', but then it

didn't feel like they really cared that it was damaging me in the process...I eventually just kept going through different psychiatrists at Kaiser because I felt like they weren't listening to me, and I just kept going through different meds and I had a bunch of different bad experiences with the meds...and then when I went to the psychiatrist about it, he prescribed me a medication that was actually known to make those worse, and I didn't find out until later because I noticed them getting worse and so then I pulled up some peer reviewed literature on it and I felt like, 'wow, that was really irresponsible to do'...I had one med that gave me a stomach ulcer, I had another med that made me suicidal, I had another medication that gave me hives and what it felt like was just that I kept going to these 15 minute appointments or 20 minute appointments that I'd have to schedule at least three months out because they never had appointments available and I'd be doing terribly those three months and then I wouldn't know how to get off the meds after they put me on them, and then after 15 minutes of talking to me like, 'oh, go try this other one after these last three gave you a stomach ulcer and made you suicidal and gave you hives and made your body ticks worse' until I was almost out of meds for me to try at Kaiser."

hence the mention of being **"poisoned"** by **"[t]heir kind"**, "their kind" referring to medical practitioners. The above excerpt mentions how my interviewee felt like they were not being listened to, and the next two lines,

“As if I were a hovel-dwelling creature, I dared to confer with my perceived saviors. In my pursuit of understanding, I soon learned of their aversion to the unknown—I have reason to suspect that they would sooner distort my story into that of a mindless beast than ever truly listen to my tale.”

refer to the dismissive and sometimes even violent way medical practitioners treat patients who advocate for themselves. The first line of this excerpt is an allusion to the scene in *Frankenstein* where the creature finally works up the courage to communicate with his perceived “saviors” (the family he has been observing during his time secretly seeking shelter in a hovel of theirs), only to be met with violence from them. The second line is a nod to the way that the media has misrepresented Frankenstein’s creature over the past two centuries since the novel was written. In the novel, the creature is an incredibly sentient, intelligent being, who attempts to advocate for his well-being while conversing with his creator. Frankenstein, the scientist who created the creature, refuses to see him as anything but an abomination. The way the media has convinced a majority of the general public to perceive Frankenstein’s creature as some zombie-like, unintelligent monster is reminiscent of how medical practitioners sometimes completely mischaracterize and misrepresent patients, convincing other medical practitioners to continue holding false narratives surrounding that patient, and improper, often harmful treatment is perpetuated as a result. An example of this is seen in Leah Ashe’s experience as she recounts in her article “From iatrogenic harm to iatrogenic violence: corruption and the end of medicine” in the way that any truth she provided in an effort to advocate for herself and any opposition toward practitioners’ false assumptions were used against her, and were eventually used to perpetuate iatrogenic violence:

“[doctor’s notes:] She feels frustrated and feels like I don’t believe her [end note]”

(Ashe 260)

“[w]ith each reinscription, the truth that I was anorexic – and its correlates, that I refused to eat, that I vomited, and that, when I said that I did eat and that I did not vomit, I was lying – became engraved ever deeper” (Ashe 261)

“The presence of (many) other contraindicating data...and the wholesale absence of concrete evidence in support of the diagnosis...did nothing to unsettle the diagnosis. It was fixed.” (Ashe 261)

“Any observant doctor should have known [the devastation this was causing], too. But these were not observant doctors; they knew everything they needed to know already.” (Ashe 263)

“that is an act reserved for people, and the psychopathology that doctors had assigned to me disqualified me from such a status.” (Ashe 264)

That last quotation can be applied to the next line,

“I finally understood that I do not fit into their classifications of human.”

which refers to the way that anyone who “fails” to fit within biomedicine’s rigidity is marked as an anomaly, and the subsequent feelings of alienation that may develop are implied with the following line,

“Am I truly so different, so abhorrent that humanity does not reach me?”

“As I continued on, doubts of my humanity grew heavier. Is my dejected state not natural for a life beyond saving? Am I unnatural? Suffering is central to human existence, yes, but in my attempted interactions with humankind I fear that my suffering may be of a kind markedly unfathomable to them.”

This excerpt presents the progression of the narrator’s doubt of their own humanity, inspired by this part of the interview:

“I felt like I was going to all of these people who had their medical degrees and had been in school for years and should know about these things that I felt like somebody should have some sort of idea of what was going on with me, and what could be wrong with me to make my life this hard to live, and then I'd be like ‘if there isn't somebody of all these people who I'm seeing—who are supposed to be professionals—if nobody knows how to help me, then why should I even try to

keep on living ‘cause it's just gonna be hard forever, and no one knows how to help.’... [my experience with medical professionals] made me feel like I was just so mentally convoluted and complex that I must be incredibly different from everyone else and too difficult to help.”

the development of suicidal ideation among individuals who experienced prolonged—especially lifelong—feelings of alienation are introduced with the following line,

“Why, then, am I expected to continue living as they do?”

and continued with the closing lines,

“The only repose guaranteed in life is death.

I am tired, and I long for rest.”

While the story I have written ends on a rather negative note, I believe that a lot of good can come from the experiences portrayed. I found Elliott Fukui’s “Stay Mad: A Love Letter to QTBIPOC Psychiatric Survivors” to be particularly inspiring. There are various intersections among the demographics who experience alienation the most, and intersectionality builds strong communities. My interviewee has not faced any challenges in seeking biomedical treatment on

the basis of race or ethnicity, but she has experienced challenges in seeking care due to her gender. Since my interviewee and I are both white European-Americans living in the United States, we do not experience any feelings of alienation derived from our race or ethnicity. However, we are both neurodivergent lesbians with chronic mental health struggles—we *do* experience feelings of alienation derived from those identities. Due to our shared experiences, we are able to confide in each other when struggling with such feelings, finding comfort in knowing that the other truly understands. We recognize that our experience is likely vastly different from that of a person of color who shares our identities, and we also recognize that our experience would likely be vastly different if we were heterosexual, neurotypical, or “mentally healthy”. With that recognition comes the opportunity to find community with others who struggle with feelings of alienation.

“As a reflection, compassionate witness, and someone who still believes we will win, I ask you to stay mad with me. Remember that when you are vulnerable and speak your truth, you are participating in changing the world. All transformation begins with us choosing to love and honor every part of ourselves and learning how to try and love every aspect of each other.

Our madness is a part of our resistance. Our failure to comply is resistance. Us choosing each other over the almighty dollar is resistance.

Stay mad, comrades, and stay together.”

— Elliot Fukui

Fukui's letter is beautiful in the way that it calls for those who have been treated as "less than a human being" to find strength in each other and embrace their identities in an act of resistance while simultaneously validating struggles faced due to those identities.

To anyone who fears they are of a markedly unfathomable kind, to anyone who longs for rest, heed the words of *Frankenstein's* "monster":

*Life, although it may only be an accumulation of anguish, is dear to me, and I will defend it.*³

³ Mary Shelley, *Frankenstein*, p. 100

Works Cited

- Ashe, Leah M. "From iatrogenic harm to iatrogenic violence: Corruption and the end of medicine." *Anthropology & Medicine*, vol. 28, no. 2, 2021, pp. 255–275, <https://doi.org/10.1080/13648470.2021.1932415>.
- Dazai, Osamu. *No Longer Human*. Translated by Donald Keene, New Directions, 2020.
- Green, Jordan, and interviewee. "Interview: A Different Kind of Medicine Story." 12 Nov. 2023.
- Shelley, Mary Wollstonecraft. *Frankenstein: Or, the Modern Prometheus*. Union Square & Co, a Subsidiary of Sterling Publishing Co., Inc, 2022.
- Thom, Kai Cheng. "The Myth of Mental Health." *DSM II: Asian American Edition*, Asian American Literary Review, 2019, pp. 2-10.
- Fukui, Elliott. "Stay Mad: A Love Letter to QTBIPOC Psychiatric Survivors." *QED: A Journal in GLBTQ Worldmaking*, vol. 9 no. 3, 2022, pp. 120-125.